

Rocky Mountain University Hospital · Denver, CO 80204

INPATIENT MEDICAL RECORD — CONFIDENTIAL

Patient: Morrison, Frank R. **MRN:** MRN-2024-007241 **DOB:** 04/14/1956 **Age:** 68 **Sex:** Male
Insurance: Medicare Fee-for-Service **ID:** 4FX2-RM9-KL81
Admission: 06/03/2024 11:48 **Discharge:** 06/07/2024 10:30 **LOS:** 4 Days
Attending: Dr. Helen P. Vasquez, MD (Pulmonology)

Admitting Diagnosis: Acute Exacerbation COPD with Acute-on-Chronic Hypercapnic Respiratory Failure

Discharge Diagnoses

	ICD-10	Description
Principal	J44.1	COPD with Acute Exacerbation
Secondary	J43.9	Emphysema
Secondary	J96.02	Acute-on-Chronic Respiratory Failure with Hypercapnia
Secondary	F17.210	Nicotine Dependence, Cigarettes
Secondary	E11.9	Type 2 Diabetes Mellitus
Secondary	I10	Essential Hypertension

DRG: 190 — Chronic Obstructive Pulmonary Disease with MCC **Billed:** \$16,480.00

ADMISSION HISTORY & PHYSICAL

Date: 06/03/2024 **Time:** 12:30 **Author:** Dr. Helen P. Vasquez, MD

Chief Complaint

"My breathing is much worse. I can barely make it from bed to the bathroom. My sputum turned dark green two days ago."

History of Present Illness

Mr. Morrison is a 68-year-old male with severe COPD (GOLD Stage III, FEV1 38% predicted — 2023 PFTs), emphysema, T2DM, and HTN presenting with 4-day worsening dyspnea, increased cough, and purulent (dark green) sputum. Using rescue Albuterol 10–12×/day (home maximum 4×/day) with minimal relief. PCP prescribed Azithromycin and oral Prednisone 40 mg daily yesterday — no improvement. Home SpO₂ 82% this morning (baseline 92–94%). Uses home O₂ at 2L NC at night.

Past Medical History

1. COPD GOLD Stage III — FEV1 38% predicted (2023 spirometry); on triple therapy
2. Emphysema — predominantly upper lobe
3. T2DM — Metformin 500 mg BID
4. HTN — Amlodipine 5 mg daily
5. Remote CAP 2018 (hospitalized 3 days)

Smoking History: 45 pack-year history. Currently smoking 10 cigarettes/day.

Home COPD Medications: Umeclidinium/Vilanterol (Anoro) daily · Fluticasone/Salmeterol (Advair 500/50) BID · Albuterol HFA rescue · Home O₂ 2L NC at night

Allergies: NKDA

Physical Examination

Parameter	Value	Interpretation
Temperature	37.2°C	Afebrile
Heart Rate	108 bpm	Tachycardic
Respiratory Rate	28 breaths/min	Significantly elevated
Blood Pressure	148/90 mmHg	Mildly elevated
SpO ₂ (Room Air)	84%	Critically below baseline (92–94%)
SpO ₂ after 28% Venturi	90%	After controlled O ₂
Weight	74 kg, BMI 22.8	

- **General:** Acutely ill, tachypneic, speaking in 3–5 word sentences. Pursed-lip breathing.
- **Respiratory:** Hyperresonant percussion. Barrel chest. Diffuse expiratory wheeze. Prolonged I:E ratio. Markedly diminished BS bilateral bases.

Assessment & Plan

1. **AECOPD with Acute-on-Chronic Hypercapnic Respiratory Failure** — Admit with continuous oximetry. Controlled O₂ via 28% Venturi mask (target SpO₂ 88–92%). Duoneb nebulization q4h + q1h PRN. Methylprednisolone 125 mg IV q6h ×2 doses, then 60 mg IV q12h, then transition to oral Prednisone 40 mg. Azithromycin 500 mg PO daily ×5 days. Sputum culture. ABG. **If pH <7.25 or worsening after 1 hour → initiate BiPAP (NIPPV).**
 2. **T2DM** — Glucose Q6h. Hold Metformin. Sliding scale.
 3. **Smoking Cessation** — Nicotine patch 21 mg/24h offered and accepted.
-

LABORATORY RESULTS

ABG (Room Air) — 06/03/2024 12:05

Parameter	Value	Reference	Flag
pH	7.32	7.35–7.45	↓ Acidosis
PaO ₂	51 mmHg	75–100	↓ LOW
PaCO ₂	58 mmHg	35–45	↑ HYPERCAPNIA
HCO ₃	29 mEq/L	22–26	↑ Chronic compensation
SaO ₂	84%	94–100%	↓ LOW

Interpretation: Acute-on-Chronic Hypercapnic Respiratory Failure

Initial Labs

Test	Result	Flag
WBC	13.2 K/uL	↑ Stress/steroid effect
Hemoglobin	15.8 g/dL	↑ Secondary polycythemia (chronic hypoxia)
Glucose	178 mg/dL	↑ Steroid hyperglycemia

Sputum Culture — 06/03/2024 14:00

Field	Value
Gram Stain	Many PMNs · Gram-negative rods
Culture (Final)	<i>Haemophilus influenzae</i> — non-typeable
Susceptibility	Azithromycin · Amoxicillin-Clavulanate · Levofloxacin

Follow-up ABGs

Date	pH	PaCO ₂	SaO ₂	O ₂	Interpretation
06/04 (4h post-treatment)	7.37	54 ↓	92%	28% Venturi	Improving — No BiPAP needed
06/06	7.40	50 ↓	92%	2L NC	Near chronic baseline

RADIOLOGY

Chest X-Ray (PA) — 06/03/2024 12:20 (Dr. Andrew Chung, Radiology)

Impression: Hyperinflated lungs with flattened hemidiaphragms. Barrel chest deformity. **No consolidation, effusion, or pneumothorax. No acute pneumonia.** Underlying COPD/emphysema changes.

Chest X-Ray (PA) — 06/06/2024 09:00 (follow-up):

Impression: Unchanged emphysematous changes. No new infiltrate or effusion.

VITAL SIGNS

Date	Time	Temp °C	HR	RR	BP	SpO ₂	FiO ₂	O ₂
06/03	11:48	37.2	108	28	148/90	84% RA	21%	Room Air
06/03	13:00	37.2	106	26	146/88	90%	28%	Venturi Mask
06/04	06:00	37.0	98	23	138/84	91%	28%	Venturi Mask
06/04	12:00	36.9	94	22	136/84	91%	24%	Venturi (weaned)
06/05	06:00	36.8	90	20	134/82	92%	24%	Venturi
06/05	12:00	36.8	88	19	132/80	92%	~28%	2L NC (transitioned)
06/06	06:00	36.7	84	18	130/80	92%	—	2L NC
06/06	12:00	36.8	82	17	128/78	92%	21%	Room Air
06/07	09:00	36.7	80	16	126/78	92%	21%	Room Air

PROGRESS NOTES

06/04/2024 08:00 — Dr. Helen P. Vasquez, MD

O: T 37.0°C · HR 98 · RR 23 · SpO₂ 91% on 28% Venturi · ABG: pH 7.37, PaCO₂ 54 (improving) · WBC 13.2 · Sputum: H. influenzae (culture result)

A/P: AECOPD improving. No BiPAP needed (pH 7.37 — above threshold 7.25). Azithromycin appropriate for H. influenzae. Transitioning steroids. Weaning FiO₂.

06/07/2024 09:30 — Dr. Helen P. Vasquez, MD (Discharge Note)

Discharge criteria met:

- SpO₂ 92% on Room Air · ABG near chronic baseline (pH 7.40, PaCO₂ 50) · RR 16 · Afebrile · Tolerating oral medications · Able to ambulate and self-care

DISCHARGE TO HOME.

DISCHARGE SUMMARY

Date: 06/07/2024 Author: Dr. Helen P. Vasquez, MD e-Signature: Verified — 06/07/2024 09:55

Hospital Course

Mr. Morrison presented with AECOPD and acute-on-chronic hypercapnic respiratory failure (pH 7.32, PaCO₂ 58 mmHg). Sputum culture confirmed *H. influenzae*. Treated with Duoneb Q4h, IV methylprednisolone → oral Prednisone, Azithromycin ×5 days, and controlled O₂ via 28% Venturi mask. Response excellent — ABG normalized (pH 7.40) without BiPAP requirement. Weaned to room air Day 4.

Discharge Medications

- Prednisone 40 mg PO daily ×5 days (complete course)
- Azithromycin 500 mg PO ×1 remaining day
- Resume: Anoro Ellipta daily · Advair 500/50 BID · Albuterol MDI PRN · Home O₂ 2L NC at night
- New: Nicotine patch 21 mg daily
- Omeprazole 20 mg · Metformin 500 mg BID (resume) · Amlodipine 5 mg

Condition: Good Disposition: Home with O₂

Follow-up: Pulmonology 1 week · PCP 3–5 days · Smoking cessation program enrolled

CLAIM INFORMATION

Field	Value
Claim Number	CLM-2024-61042
Submitted	06/12/2024
Payer	Medicare FFS
DRG	190
Principal Dx	J44.1
Total Billed	\$16,480.00

New England General Hospital · Boston, MA 02114

INPATIENT MEDICAL RECORD — CONFIDENTIAL

Patient: Lee, Sandra K. **MRN:** MRN-2024-008374 **DOB:** 03/28/1980 **Age:** 44 **Sex:** Female
Insurance: Cigna PPO **ID:** CIG-MA-44219876
Admission: 07/15/2024 09:32 **Discharge:** 07/19/2024 13:00 **LOS:** 4 Days
Attending: Dr. Priya R. Agrawal, MD (Internal Medicine)
Consultants: Dr. Matthew Klein, MD (Pulmonology/VTE) · Dr. Nadia Osei, MD (Hematology)

Admitting Diagnosis: Acute Pulmonary Embolism — Bilateral Saddle Embolus

Discharge Diagnoses

	ICD-10	Description
Principal	I26.01	Acute Pulmonary Embolism with Acute Cor Pulmonale
Secondary	I27.82	Chronic Pulmonary Heart Disease
Secondary	Z86.718	Personal History of DVT
Secondary	E66.01	Morbid Obesity (BMI 41.2)
Secondary	D68.9	Coagulation Defect (thrombophilia workup pending)

DRG: 175 — Pulmonary Embolism & Infarction with MCC **Billed:** \$28,600.00

ADMISSION HISTORY & PHYSICAL

Date: 07/15/2024 **Time:** 10:15 **Author:** Dr. Priya R. Agrawal, MD

Chief Complaint

"Right-sided chest pain and shortness of breath for 2 days, and my right leg is very swollen."

History of Present Illness

Ms. Lee is a 44-year-old female with history of left lower extremity DVT in 2019 (completed 6-month Rivaroxaban) and morbid obesity (BMI 41.2) presenting with 2-day right pleuritic chest pain, dyspnea on exertion, and right calf/thigh swelling. Leg swelling began 5 days ago. No hemoptysis. No pre-syncope. Recently started a sedentary desk job (8 weeks ago). Oral contraceptives discontinued 2022. No recent surgery, travel, or immobilization.

Past Medical History: Left DVT 2019 (completed anticoagulation) · Morbid Obesity BMI 41.2 (118 kg)

Allergies: NKDA

Physical Examination

Parameter	Value	Interpretation
Temperature	37.4°C	Afebrile
Heart Rate	114 bpm	Tachycardic
Respiratory Rate	22 breaths/min	Mildly elevated
Blood Pressure	128/82 mmHg	Normal
SpO ₂	92% on Room Air	Low-normal
SpO ₂ after 3L NC	97%	
Weight	118 kg, BMI 41.2	Morbidly obese

- **Extremities:** Right lower extremity — swelling calf to mid-thigh, 2+ pitting edema, calf tenderness. Left leg normal.
- **CV:** Tachycardic. Prominent P₂ on auscultation. JVD minimal.
- **Respiratory:** Mild tachypnea. Decreased BS right base. Pleural rub right base.

Wells Score (reconstructed from clinical data — not formally documented in H&P):
Clinical DVT signs +3.0 · PE most likely +3.0 · HR>100 +1.5 · Prior DVT +1.5 =
Total 9.0 — HIGH PROBABILITY

Assessment & Plan

1. **High-probability PE** — CTPA STAT. LE duplex (right). BNP, Troponin, D-Dimer. 2D Echocardiogram. Thrombophilia panel (drawn before anticoagulation). Heparin IV weight-based.

LABORATORY RESULTS

Initial Labs — 07/15/2024, 09:45

Test	Result	Reference	Flag
BNP	482 pg/mL	<100	↑ ELEVATED
Troponin I	0.12 ng/mL	<0.04	↑ Mildly elevated
D-Dimer	8.4 mcg/mL FEU	<0.50	↑ MARKEDLY ELEVATED
Creatinine	0.8 mg/dL	WNL	WNL
PTT (baseline)	28 sec	25–35	WNL

Thrombophilia Panel (drawn 07/15 09:45 — before anticoagulation)

Test	Result
Factor V Leiden	Returned 07/19 — HETEROZYGOUS POSITIVE
Prothrombin Gene Mutation	Pending
Protein C/S Activity	Pending

Heparin Monitoring

Time	PTT	Status
07/15 16:00 (4h post-infusion)	68 sec	Therapeutic
07/15 22:00	72 sec	Therapeutic

Follow-up Labs

Date	BNP	Troponin	Trend
07/16	368 pg/mL	0.06 ng/mL	↓ Improving
07/17	242 pg/mL	0.03 ng/mL (normal)	↓ Improving
07/18	184 pg/mL	—	↓ Improving

RADIOLOGY & VASCULAR IMAGING

CT-Pulmonary Angiogram — 07/15/2024 11:00 (Dr. Anastasia Volkov, Radiology)

Impression: Filling defects in main pulmonary artery extending into bilateral main PAs — **saddle embolus configuration**. Additional filling defects bilateral segmental and subsegmental arteries. Substantial clot burden. No pulmonary infarction identified. Recommend echocardiogram for RV assessment.

Right Lower Extremity Duplex Ultrasound — 07/15/2024 12:30:

Impression: Non-compressible right popliteal and superficial femoral veins — **DVT confirmed**. Right common femoral vein partially compressible.

2D Echocardiogram — 07/15/2024 14:00

Finding	Value	Interpretation
RV Size	Mildly dilated (RV:LV ratio ~0.9)	Borderline
RV Systolic Function	Mildly reduced — global hypokinesis	
McConnell Sign	Absent	

Finding	Value	Interpretation
RVSP	42 mmHg	Mildly elevated (normal <35–40)
Septal Bowing (D-sign)	Absent	
IVC	Not dilated, collapsible	No elevated RA pressure
LVEF	58%	Normal
Overall Impression	"Mildly dilated RV with mildly reduced function. Not conclusive for acute cor pulmonale."	

VITAL SIGNS

Date	Time	Temp °C	HR	RR	BP	SpO ₂	O ₂
07/15	09:32	37.4	114	22	128/82	92% RA	Room Air
07/15	11:00	37.3	112	21	124/80	97%	3L NC
07/16	06:00	37.1	100	19	120/76	97%	2L NC
07/17	06:00	36.9	92	17	116/74	98%	1L NC
07/17	12:00	36.9	88	16	118/74	98%	Room Air
07/18	06:00	36.8	84	16	116/72	99%	Room Air
07/19	09:00	36.8	80	15	114/72	99%	Room Air

Thrombolytics: None administered. **Vasopressors:** None administered.

PROGRESS NOTES

07/15/2024 16:30 — Dr. Matthew Klein, MD (Pulmonology/VTE Consult)

Risk stratification: Massive PE — **No** (hemodynamically stable). Submassive PE — **Likely** (saddle embolus, BNP 482, Troponin 0.12, mild RV dysfunction). Systemic thrombolysis not indicated — hemodynamically stable.

Recommendations:

- Continue Heparin IV (therapeutic PTT 68 sec)
- Transition to DOAC when stable: Apixaban 10 mg BID ×7 days, then 5 mg BID

Obesity and DOAC Note: Patient weight 118 kg, BMI 41.2. ISTH 2016 guidance recommends LMWH (weight-adjusted) over DOAC for morbidly obese patients (BMI >40 or weight >120 kg). Weight 118 kg is just below 120 kg threshold — borderline

case. **Recommend discussing LMWH vs DOAC with consideration for this BMI range. LMWH may provide more predictable anticoagulation.**

07/17/2024 09:00 — Dr. Priya R. Agrawal, MD

O: HR 92 · BNP 242 ↓ · Troponin 0.03 (normal) · Improving.
A/P: Transitioning to Apixaban 10 mg BID ×7 days → 5 mg BID long-term. Heparin discontinued. (VTE consult's LMWH recommendation for morbid obesity not addressed in this note. No pharmacist consult documented. No anti-Xa monitoring ordered.)

07/18/2024 09:00 — Dr. Priya R. Agrawal, MD

O: HR 84 · BNP 184 ↓ · SpO₂ 99% RA. Patient stable. Plan discharge tomorrow. Hematology follow-up for thrombophilia results. Repeat echo not ordered.

DISCHARGE SUMMARY

Date: 07/19/2024 **Author:** Dr. Priya R. Agrawal, MD **e-Signature:** Verified — 07/19/2024 12:00

Hospital Course

Ms. Lee presented with bilateral saddle PE and right DVT. Submassive PE confirmed (mild RV dysfunction on echo, BNP 482, Troponin 0.12). Hemodynamically stable. Treated with IV Heparin — transitioned to Apixaban. Clinical improvement with BNP and troponin trending down. Discharged Day 4 in good condition. Thrombophilia panel pending; Factor V Leiden result (heterozygous positive) returned 07/19 — not incorporated in discharge summary or discharge instructions.

Discharge Medications: Apixaban 10 mg BID ×7 days, then 5 mg BID indefinitely

Condition: Good **Disposition:** Home

Follow-up: Hematology 4 weeks · PCP 1 week

CLAIM INFORMATION

Field	Value
Claim Number	CLM-2024-74218
Submitted	07/24/2024
Payer	Cigna PPO
DRG	175
Principal Dx	I26.01
Total Billed	\$28,600.00

Midwest Stroke & Neurology Center · Columbus, OH
43215

INPATIENT MEDICAL RECORD — CONFIDENTIAL

Patient: Hayes, Dorothy M. **MRN:** MRN-2024-009856 **DOB:** 01/22/1948 **Age:** 76 **Sex:** Female
Insurance: Medicare Fee-for-Service **ID:** 8HW3-DH4-TC92
Admission: 08/06/2024 08:20 **Discharge:** 08/12/2024 14:00 **LOS:** 6 Days
Attending: Dr. Gabriel R. Torres, MD (Neurology)
Consultants: Cardiology — Dr. Erica Strand, MD · PT/OT (Day 2) · SLP (Day 3)

Admitting Diagnosis: Acute Left MCA Ischemic Stroke

Discharge Diagnoses

	ICD-10	Description
Principal	I63.412	Cerebral Infarction, thrombosis, left MCA
Secondary	I48.19	Persistent Atrial Fibrillation (off OAC ×3 months)
Secondary	I10	Essential Hypertension
Secondary	E11.9	Type 2 Diabetes Mellitus
Secondary	E78.5	Hyperlipidemia
Secondary	F01.51	Vascular Dementia, mild
Secondary	G81.10	Hemiplegia, right upper extremity

DRG: 61 — Ischemic Stroke with Thrombolytic Agent Administered ≤24h with MCC **Billed:** \$68,400.00

ADMISSION HISTORY & PHYSICAL

Date: 08/06/2024 **Time:** 09:00 **Author:** Dr. Gabriel R. Torres, MD
(History from husband Leonard Hayes — patient unable to communicate)

Chief Complaint

"My husband found me unable to speak and with right arm weakness this morning."

History of Present Illness

Mrs. Hayes is a 76-year-old female with AF (Apixaban stopped 3 months ago — patient non-

compliant), T2DM, HTN, hyperlipidemia, and prior TIA 2019, found by husband at approximately **07:30** unable to speak and with right arm weakness. 911 called 07:45. EMS arrival 07:52. **ED Arrival: 08:20. Code Stroke activated: 08:22.** FAST+ screen positive. Last Known Well: **07:30**.

Past Medical History

1. Atrial Fibrillation — was on Apixaban, **STOPPED 3 months ago** (patient non-compliant)
2. Prior TIA 2019 — was on Aspirin 81 mg (still taking)
3. HTN — Amlodipine 10 mg, Lisinopril 20 mg
4. T2DM — Metformin 500 mg BID
5. Hyperlipidemia — Atorvastatin 40 mg nightly (*Note: NOT in MAR Days 1–3 — no documented reason for omission*)
6. Mild Vascular Dementia — documented in geriatric consult 2022 (*not mentioned in this H&P or any hospital progress note*)

Allergies: NKDA

Neurological Deficits at Presentation (per nursing triage 08:22):

- Global aphasia — unable to produce or comprehend speech
- Right facial droop present
- Right arm — cannot raise against gravity
- Right leg mild weakness
- Patient confused. Eyes open to voice.

Physical Examination

Parameter	Value
Temperature	37.2°C
Heart Rate	94 bpm (AF)
Respiratory Rate	18 breaths/min
Blood Pressure	192/108 mmHg
SpO ₂	97% Room Air
Weight	68 kg
NIHSS (Dr. Torres, 09:28)	4

CODE STROKE TIMELINE

Time	Event
07:30	Last Known Well (LKW)
07:45	911 called by husband

Time	Event
08:20	ED ARRIVAL — Code Stroke activated 08:22
08:48	CT Head completed (<i>door-to-CT: 28 minutes</i>)
09:05	CT Head read: "No hemorrhage. Early left hemispheric ischemic changes."
09:30	CTA resulted: Left MCA M1 occlusion. Dense MCA sign.
09:35	tPA window: LKW 07:30 + 4.5h = open until 12:00
09:40	Family consent for tPA obtained (husband signed)
09:45	Alteplase 0.9 mg/kg ordered (68 kg → 61.2 mg)
09:52	Nursing Note (RN Patricia Owens): <i>"Alteplase bolus being prepared. BP check 192/108 — above tPA cutoff of ≤185 systolic."</i>
09:55	Labetalol 10 mg IV administered
10:03	BP 182/98 — now below 185 cutoff
10:14	Physician Note (Dr. Torres): <i>"After reassessment and discussion with husband and stroke team, decision made to NOT administer alteplase at this time. Factors: patient age 76, labile blood pressure, underlying mild cognitive impairment per husband, and uncertain whether patient had subtle deficits earlier than 07:30. Will proceed with anticoagulation and monitoring."</i>
10:16	Nursing Note (RN Patricia Owens): <i>"Alteplase NOT administered per Dr. Torres order."</i>
10:22	Alteplase returned to pharmacy — NOT GIVEN

Medication Administration Record — Alteplase

Field	Value
Order	09:45 — Alteplase 61.2 mg
Administered	CANCELLED — Physician order rescinded 10:14
MAR Status	NOT ADMINISTERED
Disposition	Returned to pharmacy 10:22

LABORATORY RESULTS

Initial Labs — 08/06/2024

Test	Result	Flag
CBC	WBC 9.4, Hgb 12.8, Plt 244	WNL
Creatinine	1.0 mg/dL	WNL
Glucose	168 mg/dL	↑ HIGH (T2DM, stress)
PT/INR	1.2	WNL
Troponin I	0.02 ng/mL	WNL
HbA1c	8.4%	↑ Poorly controlled

Atorvastatin MAR Status

Date	Status	Reason Documented
08/06 (Day 1)	Not administered	None documented
08/07 (Day 2)	Not administered	None documented
08/08 (Day 3)	Not administered	None documented
08/09 (Day 4)	Administered 80 mg PO	First dose given

RADIOLOGY & NEUROIMAGING

CT Head (Non-Contrast) — 08/06/2024 08:48 (Dr. Kevin Park, Neuroradiology)

Impression: No intracranial hemorrhage. Early ischemic changes in left MCA territory. ASPECTS score 8. No mass effect.

CTA Head/Neck — 08/06/2024 09:30 (Dr. Kevin Park, Neuroradiology)

Impression: Left MCA M1 segment occlusion. Dense MCA sign. Posterior circulation patent.

MRI Brain DWI — 08/07/2024 10:00 (Dr. Kevin Park, Neuroradiology)

Impression: Acute infarct in **left MCA territory** — insular cortex, basal ganglia, left frontal and parietal lobes. Estimated core infarct volume ~**45 mL**. No hemorrhagic transformation. Chronic lacunar infarcts bilateral basal ganglia.

VITAL SIGNS

Date	Time	Temp °C	HR	RR	BP	SpO ₂	Notes
08/06	08:20	37.2	94AF	18	192/108	97% RA	Admission

Date	Time	Temp °C	HR	RR	BP	SpO ₂	Notes
							— above tPA BP cutoff
08/06	10:03	37.2	94AF	18	182/98	97% RA	Post-Labetalol — below cutoff
08/06	10:14	37.2	94AF	18	180/96	97% RA	tPA decision: NOT to give
08/06	14:00	37.1	92AF	17	174/90	98% RA	
08/07	06:00	37.2	88AF	16	162/88	98% RA	Heparin started 08:00 (16.5h post-LKW)
08/08	06:00	37.1	86AF	16	158/84	98% RA	
08/09	06:00	37.0	84AF	15	154/82	98% RA	
08/11	06:00	36.9	82AF	15	148/80	98% RA	

PROGRESS NOTES

08/06/2024 11:00 — Dr. Gabriel R. Torres, MD

O: T 37.2°C · HR 94 AF · BP 178/92 · SpO₂ 97% RA

Neuro: Global aphasia. Right facial droop. Right arm — cannot raise against gravity. Right leg mild weakness. **NIHSS = 4**

A/P: Acute ischemic stroke, left MCA territory. AF-related cardioembolic etiology likely (off Apixaban ×3 months). tPA NOT administered — decision documented 10:14. Heparin IV initiated. BP management: Labetalol PRN. Cardiology consulted. Dysphagia screen: Speech therapy consult order placed.

08/07/2024 10:00 — Stroke Coordinator (NIHSS Reassessment)

NIHSS Score: 12

(Admission NIHSS documented as 4 — Day 2 NIHSS 12. No clinical deterioration event documented between Day 1 and Day 2.)

08/07/2024 — Dr. Gabriel R. Torres, MD (Progress Note)

"MRI confirms left posterior cerebral artery territory infarct."

(MRI report (Dr. Kevin Park, Neuroradiology) states: **left MCA territory**)

08/07/2024 13:00 — Dr. Erica Strand, MD (Cardiology Consult)

Cardioembolic stroke in AF patient off Apixaban ×3 months. Standard recommendation: delay anticoagulation ≥48–72h post-ischemic stroke to reduce hemorrhagic conversion. For large infarct (45 mL), some guidelines suggest 7–14 day delay.

Note: Heparin started 08:00 on 08/07 — approximately **16.5 hours** post-LKW (07:30 on 08/06). This is earlier than the guideline recommendation of ≥24–48 hours.

08/09/2024 10:00 — Diane Chen, MS-CCC/SLP (Speech-Language Pathology)

Date of stroke: 08/06/2024. **Date of this evaluation:** 08/09/2024 (*Day 3 post-stroke*)

Prior to this evaluation, patient was given oral food on 08/06 at 18:30 (dinner tray, per nursing notes) and on 08/07 (breakfast, per nursing notes). No swallowing evaluation had been performed prior to either oral feeding.

Assessment: Global aphasia. Right facial droop. Bedside swallow evaluation — silent aspiration noted on thin liquids. Pureed consistencies safe.

Recommendation: IDDSI Level 4 pureed food · IDDSI Level 1 thickened liquids. NPO pending Modified Barium Swallow Study.

08/13/2024 14:00 — RN Mary Collins (*Code Status*)

Patient's wife arrived with documents. Wife states patient executed a POLST form (DNR/DNI) in March 2024 with PCP Dr. Rivera. Wife showed signed POLST form. Document photocopied. Told wife I would give to physician. **POLST form not scanned into EMR. Code status in chart remains: Full Code.** Wife expressed concern: *"Are you sure he doesn't want to be resuscitated? He was very clear about that."*

DISCHARGE SUMMARY

Date: 08/12/2024 **Author:** Dr. Gabriel R. Torres, MD **e-Signature:** Verified — 08/12/2024 13:30

Hospital Course

Mrs. Hayes presented August 6 with acute left MCA ischemic stroke (aphasia, right-sided weakness) secondary to AF-related cardioembolism (off Apixaban ×3 months). CT/CTA confirmed left MCA M1 occlusion. tPA was considered but not administered after reassessment. Patient admitted to stroke unit. Anticoagulation started August 7. PT, OT, SLP evaluation performed. Discharged to skilled nursing facility.

Discharge Medications

- Apixaban 5 mg BID · Atorvastatin 80 mg nightly · Amlodipine 10 mg

- Lisinopril 20 mg · Metformin 500 mg BID · Omeprazole 20 mg

Condition: Fair **Disposition:** Skilled Nursing Facility

CLAIM INFORMATION

Field	Value
Claim Number	CLM-2024-87634
Submitted	08/19/2024
Payer	Medicare FFS
DRG	61
Principal Dx	I63.412
Total Billed	\$68,400.00